

CHIROPRACTIC SLEEP ACADEMY®

The Workbook

*Bridging Biomechanics, Airway Science, and Sleep Medicine for the
Doctor of Chiropractic — Credible Edition*

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www.ChiropracticSleepAcademy.net

**Continuing Education approved by the California Board of Chiropractic
Examiners**

FRONT MATTER

About This Edition, Disclaimers & Foundation

How this edition works. This is the credible, professional edition — the biomechanical spine, governed by CSA's rule: *we judge clinical consequence, not truth claims*. Every model is labeled **established** or **hypothesis under investigation**; outcomes are measured before/after and reported honestly, including non-responders. Energy/frequency content and product endorsements are kept out of the professional curriculum. Faith is honored in one clearly-marked Foundation section with a secular twin, so the science stands on its own.

Disclaimers

This workbook is **education** for Doctors of Chiropractic — not individualized medical advice and not a diagnosis of any individual reader or patient. In California the Doctor of Chiropractic **diagnoses and manages** sleep apnea, snoring, and insomnia within full scope: the DC reads the case clinically and directs it, and **co-manages complex cases with an MD/ENT as peers, by choice**. Nothing here positions structural care as a replacement for CPAP, oral appliances, surgery, or medication, and no patient is advised to stop prescribed therapy.

FOUNDATION — FOUNDER'S PHILOSOPHICAL BASIS (OPTIONAL TO THE SCIENCE)

Two registers. The clinical science in this book stands entirely on its own — no belief required. This section states the founder's personal, faith-based *why*; it is not part of the clinical argument.

Secular statement: The body is a living, whole system worth caring for with rigor and respect; restoring structure so a person can sleep and breathe well is meaningful clinical work.

Founder's faith foundation (Dr. Zeravica): *"Know ye not that your body is the temple..."* (1 Cor. 6:19–20); *"...present your bodies a living sacrifice..."* (Rom. 12:1). Caring for the physical structure as a living temple is stewardship — honoring design by restoring alignment and unobstructed flow so the whole person can function well.

Mission

To equip Doctors of Chiropractic to **raise awareness, screen, order and interpret testing, diagnose, educate, manage and plan structural care, co-manage, and re-evaluate** patients with airway-related sleep concerns — restoring structure so sleep, breathing, and function can improve, always measured and always within scope.

About the Author

VERIFY CREDENTIALS BEFORE PRINT

Dr. Robert Zeravica earned a B.S. in Business Administration (Pepperdine), and his Doctor of Chiropractic from the Los Angeles College of Chiropractic (1999); active California license. A U.S. Marine Corps NCO, he served in 4th Force Reconnaissance and 3rd ANGLICO (Air Naval Gunfire Liaison Company) — the liaison and precision-coordination skills that now shape his interprofessional work across medicine, dentistry, and chiropractic. He is balloon-instrument trained in Cranial Facial Release (CFR) and Bilateral Nasal Specific (BNS) and is the developer of VAET™.

PART I

Foundations of Structural Airway Chiropractic

Ch 1 — History & Philosophy

Chiropractic began in the late 19th century on the principle that **structure governs function**. D.D. Palmer founded it alongside an era rich in anatomy, neurology, and vitalism; A.T. Still founded osteopathy in the same period. CSA is **chiropractic** — it studies the whole body and learns freely from any tradition, but is not "osteopathic." California matured chiropractic into a licensed, diagnosing profession within its defined scope.

Ch 2 — From Dogma to Discernment GOVERNING METHOD

CSA's operating rule, plainly: **we do not judge truth claims; we judge clinical consequence**. A technique earns standing by what it measurably produces, not by the confidence of its story. Hold strong roots, test everything, keep what works, and report honestly — including what doesn't respond.

Ch 3 — Innate Intelligence & Primary Respiratory Mechanism

Innate intelligence — the body's organizing, self-healing capacity — is expressed in measurable ways (cellular signaling, tissue adaptation, neural plasticity); taught as biology, not mysticism. **PRI/PRM** and the **Reciprocal Tension Membrane / dura** are presented as cranial-osteopathic *models under investigation* — useful lenses, not settled physiology.

Ch 4 — Anatomy for Airway Function ESTABLISHED

- **Viscerocranium & sphenoid** — facial skeleton and keystone bone shaping airway aperture and sinus drainage.
- **Hyoid apparatus** — suspends tongue and larynx; governs tongue posture and airway patency.
- **Mandibular plane** — a measurable indicator of craniofacial growth and airway volume.

Ch 5 — Tensegrity / Biotensegrity CORE FRAMING

Tensegrity (Fuller; Snelson) and its biological application **biotensegrity (Stephen Levin, MD)** describe the body as a pre-stressed, load-sharing network: bones float within a continuous tension web. CSA **applies** this established framing to the cranium and airway — *we did not invent it*. The teaching phrase: **motion is distributed, deformation is shared; no single joint "moves" — the system yields**. Suture micromotion and compliance under load are real; the degree and its sleep/airway significance are what CSA is testing. The credible replacement for "cranial pump" language.

PART II

Sleep Science & Airway Mechanics

Ch 6 — Upper-Airway Obstructive Sleep Apnea (UAOSA)

Mechanical collapse of the upper airway during sleep → intermittent desaturation, micro-arousals, systemic stress. CSA's contribution is a **biomechanical lens**: craniofacial restriction, TMJ/hyoid imbalance, forward-head posture / cervical hypolordosis, and pelvic compensation as possible *contributors* — in addition to, not instead of, the recognized medical picture.

Ch 7 — Sleep Homeostasis MAINSTREAM

Adenosine accumulates during wakefulness and builds sleep pressure. (*A proposed hypothesis that adenosine concentrates in cranial/sphenoid structures is unproven — present it as a question to investigate, never as fact.*)

Ch 8 — Testing & Severity HST + CLINICAL READ

The **Home Sleep Test (HST)** records O₂ saturation, airflow, snoring, and AHI — ordered through a licensed HST pathway; the **DC reads the case clinically and diagnoses** it. AHI (AASM): Normal <5 · Mild 5–14 · Moderate 15–29 · Severe ≥30. The DC correlates numbers with structural findings within scope.

Ch 9 — The Consequence Standard in Practice

Document **before and after**: posture photos, CBCT/lateral cephalometric airway volume, HST metrics (AHI/ODI/SpO₂), patient-reported outcomes. Report **responders and non-responders**. Improvement shows structure *contributed in that case* — never a cure, never proof of mechanism. Data → the CSA Outcomes Registry.

Ch 10 — Report of Findings, Education & Scope

Core scope rule: **sleep and airway are biomechanical problems that cross licensure lines; CSA is where those lines collaborate without collapsing scope**. Screen, coordinate testing, educate, co-manage, refer.

PART III

VAET™ as a Framework

Ch 11 — What VAET Is

VAET is **not a new cranial technique** but a **condition-specific application framework** sequencing established methods (CFR/BNS, SOT, AK, CBP, postural-foundation work) for the airway indication, in a defined order and time-limited window. Offered as a legitimate **early structural intervention, honestly framed** — always measured (pre/post HST), co-managed, and **never** positioned as superior to or a replacement for CPAP, appliances, or surgery.

Ch 12 — CFR / BNS WITH SAFETY

CFR mobilizes cranial sutures via the nasal passages (Dr. Z certified since 2012), within a full-body structural model; BNS is a related nasal/cranial method. **Safety:** sterile nasal balloons only; contraindications include recent nasal surgery, severe sinus infection, uncontrolled hypertension; obtain informed consent; document nasal patency pre/post.

Ch 13 — SOT, CBP & Postural Foundation

SOT pelvic categories I-III and CBP cervical-lordosis correction are integrated and **measured** (e.g., pre/post cervical angles). Any claim that lordosis change reduces AHI is a *hypothesis to test per case*, reported honestly — not a promised result.

Ch 14 — Cerebrospinal Fluid

Teach precisely: **CSF exchange is rhythmic and sleep-enhanced** (pulsatile, glymphatic — established). Whether **cranial mechanics assist** it is *CSA's model under investigation*. State the two clauses separately. (*"Liquid Light"/"trampoline effect" are internal metaphors only — not professional physiology.*)

Ch 15 — Documentation

SOAP notes; VAET session findings; pre/post metrics; re-evaluation timeline. Documentation converts belief into evidence.

Whole-Person Support

Ch 16 — Lifestyle Foundations EVIDENCE-BASED ONLY

Sleep, nutrition, movement, weight where relevant, hydration, and stress each influence airway and recovery. Teach the evidence-based basics (e.g., NEW START / lifestyle-medicine principles as habit prescriptions), measured like everything else.

What was removed, and why. The prior draft's energy/frequency "medicine" (scalar-energy devices, Rife machines, "electromagnetic vitality," ozone otic/rectal insufflation, alkaline/deuterium-depleted water, colloidal products) and its commercial product endorsements are **removed from the professional CE curriculum**. They read as pseudoscience to the MD/DDS/college audience CSA needs, some carry safety/regulatory concerns, and endorsements create conflict-of-interest exposure. Any personal product use belongs in a separate, clearly-labeled resource **with disclosure** — never inside the board-approved clinical curriculum.

PART V

Applied Clinical Protocols

Ch 17 — Screening & Patient Education

Mallampati class as an **education/communication tool (not a diagnostic test)**; snoring/witnessed-apnea logs; STOP-BANG/Epworth screening; the structural airway exam (Mallampati, tongue scalloping, forward-head, cranial- postural symmetry, **asymmetric palatal yield** as a documented observation to correlate — not a diagnosis).

Ch 18 — Case Documentation & the Registry

De-identified case records (Case IDs, no PHI in shared trackers); outcomes reported in full including non-responders; feeding the CSA Outcomes Registry — the honest evidence base that earns the profession's respect.

Ch 19 — Scope & Referral Logic

When to screen, test, co-manage, refer (MD/ENT/DDS). Every provider stays in their lane; CSA coordinates. Moderate-severe → medical co-management.

PART VI

Professional Development

Ch 20 — The CSA Continuing-Education Model

CE approved by the **California Board of Chiropractic Examiners** (state-specific).

Competencies: screen; interpret HST in a biomechanical context (within scope); design a co-managed structural plan; document outcomes. Formats: intensives, online modules, mentorship study groups.

Ch 21 — Ethical Growth & Collaboration

Marketing is the delivery system for a true message, not manipulation. Build relational capital with clinical, educational, and business allies. *(Named mentors/influences described accurately — "studied their work" vs. personal mentorship — no endorsement implied without consent.)*

Ch 22 — Outcomes, Recognition & Future

Publish de-identified outcomes; pursue academic/CE recognition on the strength of **data**, not claims; expand tracks as evidence supports.

Appendices and Editor's Checklist

Appendices (keep): A. Foundation (faith, marked) · B. Mallampati chart (education, not diagnosis) · C. Postural analysis checklist · D. SOT pelvic- category flow chart · E. Structural airway exam form (incl. asymmetric palatal- yield observation) · F. Pre/Post outcomes worksheet (AHI/ODI/SpO₂/Epworth/airway volume; responder + non-responder fields) · G. References (AASM; craniofacial-OSA literature; Levin biotensegrity; glymphatic/CSF-in-sleep literature).

Editor's checklist before print

- Verify every credential/abbreviation against certificates.
- State CE approval as "California Board of Chiropractic Examiners" (per-state).
- Confirm no energy-medicine / product-endorsement content re-entered the CE text.
- Every mechanism labeled established vs. hypothesis; no cure claims; no "instead of CPAP."
- One brand + domain: Chiropractic Sleep Academy™ / .net.
- Attorney review of scope language and any liability/CE statements.